جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : نعمه ابراهيم السيد السيد
المحافظة : القاهره **الحي / المدينة :** الشرابيه **العنوان :** -----
رقم البطاقة : 27706170103001 **رقم التليفون :** 01254876589 **تاريخ تسجيل الإستمارة :** 11-10-2026
العمر(تاريخ الميلاد) : 17-06-1977 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج الالم ومضادات القئ لمرضي الاورام المنتشر [14456] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: نعمه ابراهيم السيد السيد
National ID: 27706170103001

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

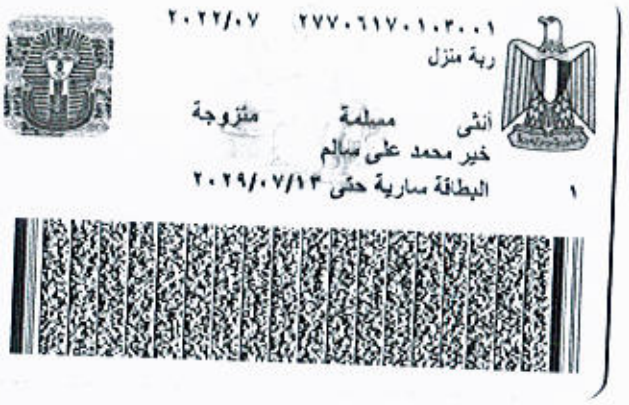
نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Pin Medication And Anti Emetics

التاريخ : 05-07-2026	**الوقت :** -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

GUSTAVE ROUSSY INTERNATIONAL EGYPT

MDT FORM

Patient Name	نعمة ابراهيم السيد	Age	46
Sex	F	Special habits	
Family History		Comorbidity	
Surgical History		Telephone	0115
Menopausal Status	Premenopausal	Marital Status	
ID	27706170103001	Medical ID	90/2024
Date	12/10/2025	Reason of MDT	

Medical Report

Pathology: Bx-> LT IDC II,
ER 7/8, PR 3/8 Her2: -ve
PS 1

CT4b N3 M0
Rec NACT 4AC + 4 taxotere- Clinically responding
Lt MRM: free resection margins, Free skin
LN 2/10
ypT2(3x2.5cm) N(2/10) M0 (extra capsular extension), Pt for Port, Adjuvant (AI + Zoladex) x 5 years.
S.M: Lt breast multifocal suspicious neoplasm with ipsilateral pathological axillary and supraclavicular lymphadenopathy (Birads 4C)
Metsworkup: non contrast CTCAP (18/8/2025) Peritoneal nodules , Left mastectomy with suspected nodular thickening, multiple mediastinal L.Ns likely reactive, left upper lobe radiation pneumonitis, Newly developed small left subphrenic nodule 1x1cm
Biopsy not visible
Recommendations

MDT DECISION	For cdk46+ faslodex +zoladex Zometa every 6 month

Name	دد/ حنان عزت	د/ رانيا شفيق	
Signature			

Name: نعمه ابراهيم السيد السيد	
MR. : 308	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 48-year-old female with a history of left breast invasive ductal carcinoma, grade II, ER 7/8, PR 3/8, HER2-negative, staged T4BN3M0. She received adjuvant chemotherapy consisting of 4 cycles of doxorubicin/cyclophosphamide (AC) followed by 4 cycles of docetaxel (Taxotere). A left modified radical mastectomy was performed in September 2024, with pathology revealing 2/10 positive lymph nodes, 3 × 2.5 cm tumor, and extra-capsular extension. The multidisciplinary team recommended a left chest wall, internal mammary, and axillary radiation field, and initiation of adjuvant endocrine therapy with an aromatase inhibitor (AI) plus zoledronic acid for five years, starting in October 2024. Imaging in August 2025 showed a left upper lobe radiation pneumonitis and a newly developed 1 × 1 cm small left subphrenic nodule. The plan was to initiate Ibrance (palbociclib) plus fulvestrant and zoledronic acid pending approval, with a 3-month course of Ibrance and fulvestrant scheduled.

The patient is currently receiving fulvestrant (Faslodex) and zoledronic acid, with Ibrance therapy pending approval. No cycle or week markers are documented in the notes, so the current status is that the patient is awaiting initiation of the planned CDK4/6 inhibitor regimen.

تحريرا في ,

التاريخ :	**20-09-2026**	**الوقت :**	

الطبيب المعالج : **ايه مدحت بدوي محمد**

يعتمد ,

Name : Naima Ibrahim El-Sayed
Branch : Dokki

Date : 03-12-2025 10:35
Medical No. : 262528
Ins.Company : Walkin

NILE DIAGNOSTIC CENTER

$Tc^{99}m$ MDPSKELETAL SCINTIGRAPHY

History

- A 48-year-old lady, with history of left breast cancer and referred for metastatic workup.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings

- No abnormal active or cold focal bony lesions that could account for metastatic bone disease.
- Degenerative changes are noted at the spine and big joints.

Impression:

- **The skeleton is free from metastases.**

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

Patient Name	: الاستاذه / نعمه ابراهيم السيد السيد	Request No.	: 13008221
Patient No.	: 2770617010300	Request Date	: 10/01/2024
Path No.	: SR-33/24	Verification Date	: 22/01/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 46 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer.
Nature of Specimen: Slide Revision.

Gross Examination:
- One paraffin block labeled by (9205/23) and three slides were received.

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade II (3+2+1).
* DCIS: Focal.
* IPT:
- ER: Moderate nuclear reaction in 80% of tumor cells (Allred score 7/8).
- PR: Weak nuclear reaction in 3% of tumor cells (Allred score 3/8).
- Her2: Faint incomplete membrane reaction in >10% of tumor cells (score 1+).

Diagnosis:
Left breast cancer, Slide revision:
Invasive duct carcinoma, Grade II with focal DCIS.
IPT:
- ER: Positive (score 7/8).
- PR: Positive (score 3/8).
- Her2: Score 1+.

Signature
Dr. Rana Fathy
Dr. Toka Medhat

Verified by :
Dr.Marwa mahmoud

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

CAP ACCREDITED
Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / نعمه ابراهيم السيد	Request No.	: 13002061
Patient No.	: 27706170103001	Request Date	: 21/09/2024
Path No.	: 3535/24	Verification Date	: 29/09/2024
Sex	: Female	Financial Category	: ورادة دار السلام - مبادرة
Age	: 44 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, post NACT.
Nature Specimen: Left modified radical mastectomy specimen.

Gross Examination:
* Procedure: Left MRM measured 28x27x6 cm, with covering skin ellipse measured 28x26 cm.
* Tumor extent: A central greyish white tumor bed site.
* Size: 3x2.5x1 cm.
* Skin: Nipple & areola looked unremarkable grossly.
* Additional findings: Rubbery grey white area 8x7x5 cm.
* Distance from margins: 2 cm away from the deep margin.
* Lymph nodes number and size: 10 L.Ns up to 4x2.5x2.5 cm.

- 13 Blocks:
1:3- T.B.S., 4:6- G/w area., 7- N&A + skin., 8- Deep margin., 9:13- L.Ns.
- Gross was done by; Dr. Esraa Magdy.

Microscopic Examination:
* Histological type: Residual viable remnants of invasive duct carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+2) Grade II.
- Maximal tumor diameter 3x2.5 cm.
- Actual tumor cellularity: 20%.
* Ductal carcinoma in situ: Not detected.
* Lympho-vascular invasion: Extensive.
* Perineutral invasion: Not detected.
* Skin: Free with free N&A.
* Resection Margins: Free.
* Lymph nodes: Positive metastatic deposits in two out of ten lymph nodes (The largest

Verified by :
Dr.Duaa Mohamed

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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الاستاذه / نعمه ابراهيم السيد	Request No.	: 13002061
Patient No.	: 27706170103001	Request Date	: 21/09/2024
Path No.	: 3535/24	Verification Date	: 29/09/2024
Sex	: Female	Financial Category	: اورنج دار السلام - مباشره
Age	: 44 Y	Department	: Out Patient
Referreing Doc :		Registered by	: marwa mohamed youss

Pathology Report

nodal metastasis is 0.3x0.3 cm), with evidence of extra-nodal extension.
*.Histologic response to therapy: Pathological partial response to therapy.

Diagnosis:
Left breast cancer post NACT, Left modified radical mastectomy specimen:
Residual viable invasive duct carcinoma, Grade II.
- Maximal tumor diameter: 3x2.5 cm.
- Actual tumor cellularity: 20%.
- Resection Margins: Free.
- Skin: Free.
- Lymph nodes: Positive metastatic deposits in two out of ten lymph nodes (The largest nodal metastasis is 0.3x0.3 cm), with evidence of extra-nodal extension.
- Histologic response to therapy: Pathological partial response to therapy.
- Stage: ypT2, ypN1a.
- RCB: 3.195, RCB class II.

Signature
Dr. Rana Fathy.
Dr. Duaa Mohamed.

Verified by :
Dr.Duaa Mohamed

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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

PATIENT NAME: NEAMAA IBRAHIM ELSAYYED ELSAYYED
GENDER: Female
DATE : 20-Dec-2023 11:27 AM

PATIENT ID: 10123236532
AGE: 46 y
ORDER: 2635696,2635697

Clinical indication - all studies
Left breast neoplasm

Report: - all studies

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

TECHNIQUE:

- CC and MLO view.
- Complementary ultrasound

REPORT:

- Scaltered fibroglandular densities (ACR B).
- No right spiculated mass lesions or grouped microcalcifications.
- **Left retroareolar ,UOQ global assymtry of increased overall density obscuring underlying lesions associated with retracted nipple and edema pattern ,skin thickness up to 7mm**
- **Complementary ultrasound depicted para areolar 12 o;clock and 3 o;clock irregular hypoechoic mass lesions with speculated margins ,the largest measuring 2x1.6 cm**
- No right solid nor cystic lesion .
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Right Nonspecific axillary lymph nodes.
- **Left multiple pathological axillary lymphadenopathy (LEVEL I,II,III) with few supraclavicular lymphadenopathy**

OPINION:

Left breast : picture keeping with multifocal suspicious neoplasm with ipsilateral pathological axillary and supraclavicular lymphadenopathy as described for histopathological correlation (BIRADS 4C)

- **Right breast :** Fibroadenotic mammary changes (BIRADS 2).

Recommendation - all studies
Referral to clinic

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مستشفى بهية للشيخ زايد
٢ الحي الأول الشيخ زايد أمام كلية الهندسة
Mammogram
[[DOCTYPE:MAMMOGRAM]]

GUSTAVE ROUSSY
INTERNATIONAL
EGYPT

Patient Name: Ne'ma Ibrahim Al Sayed	Date : 18/08/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 48 Year(s)
Medical ID: 90/2024	Pat. ID: 46473

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

I Chest:-

- Left mastectomy with suspected nodular thickening for sonomamographic correlation.
- No sizable axillary lymph nodes .
- Multiple mediastinal L. Ns , the largest at prevascular space mesures 12x 7 mm.
- Still noted few millimetric left supraclavicular lymph nodes.
- Left upper lobe subpleural reticulation likely post radiotherapy.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Thyroid isthmic hypodense nodule measuring 2 x 1.4 cm for US correlation.

II Abdomen & pelvis:

- The liver is mildly enlarged with homogenous CT texture. No focal lesions or contour irregularities on non contrast basis.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- **Newly developed Small left subphrenic nodule 1 x1 cm.**
- **Right iliac fossa multiple peritoneal nodules , the largest measures 18 x12 mm.**
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- No suspcious osseous lesions.

Opinion:

- **Left mastectomy with suspected nodular thickening.**
- **Multiple mediastinal L. Ns likely reactive .**
- **Left upper lobe radiation pneumonitis.**
- **Newly developed Small left subphrenic nodule 1 x1 cm.**

- **Right iliac fossa multiple peritoneal nodules , the largest measures 18 x12 mm.**
- **Rather stationary rest of the study.**

Dr. Reham Khedr, M.D.

Signature Reeham.Khader
22/08/2025 12:16 PM